Manson Public Health Consultancy Group Ltd.

DELIVERABLE 5: UPDATED NATIONAL HIV DISCLOSURE PROTOCOL

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National HIV/STI Programme

National HIV Disclosure Protocol

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The National HIV Disclosure Protocol stemmed from a wide consultative process with healthcare providers across the four (4) Regional Health Authorities as well as the wider network of HIV service providers in the NGO and advocacy landscape. These consultations involved varied categories of staff working in prevention, treatment care and support as well as PLHIV who access services through the national network of HIV providers. The MOHW wishes to acknowledge the contribution of these personnel and PLHIV who have actively provided input to this protocol.

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Acronyms and Abbreviations

ARV - antiretroviral

ART - antiretroviral therapy

AHF - AIDS Healthcare Foundation

GBV - gender based violence

HCP - healthcare providers

HIV - Human Immunodeficiency Virus

JASL - Jamaica Aids Support for Life

JN+ - Jamaica Network of Seropositive

LGBTQ - lesbian, gay, bisexual, transexual, queer

NDP - National Disclosure Protocol

NERHA - North East Regional Health Authority

NGO - non-governmental organization

PATH - Programme of Advancement Through Health and Education

PLHIV - persons living with HIV

SRH - Sexual and Reproductive Health

SRHA - Southern Regional Health Authority

SERHA - South East Regional Health Authority

TCS - Treatment Care and Support

WRHA - Western Regional Health Authority

SECTION I: OVERVIEW

About the Protocol

The National Disclosure Protocol (NDP) was developed to address gaps identified by Health Care Providers (HCP) in the National HIV Programme, Non-Governmental Organizations (NGO) and other HIV service providers, in assisting to guide Persons Living with HIV (PLHIV), to disclose their status, to partners, close friends, family, and others they choose to. This protocol seeks to provide an easily accessible reference that can be utilized by the network of HIV care providers across the country.

Purpose

The objective of the NDP is to provide a standardized approach to HIV status disclosure for adults linked to care. The document contains tools that can be used by HCP to assist in facilitating informed disclosure from pre-disclosure assessment to post-disclosure support.

Target Audience

The NDP is geared towards HCP who work with adult PLHIV linked to care. The category of HCP include: Social Workers, Case Managers, Adherence Counselors, Psychologists, Nurses, Physicians, Behavior Change Coordinators, Peer Navigators, and other relevant members of the treatment, care and support (TCS) team.

How to Use the Protocol

The NDP is divided into four (4) main sections:

a. The Background which outlines the importance of disclosure, types of disclosure, benefits, challenges, and barriers to disclosure.
b. The Process of Disclosure that summarizes the general principles of disclosure, special considerations, planning for disclosure and the stages of disclosure.

c. Tools which help the HCP to carry out pre-disclosure assessment as well as the follow up and support after disclosure.
d. Appendix that contains algorithms that will assist HCP to recall the major components of the protocol.

Users should ideally go through the content of the NDP in its entirety to gather a greater understanding of the relevant issues surrounding disclosure.

Limitations of the Protocol

The protocol seeks to address the major aspects in the process of disclosure and provides the broad context for dealing with the challenges faced by adult PLHIV. The protocol also addresses special considerations that are to be made for certain marginalized and key groups of PLHIV who access care through the PLHIV care provider network. Even though the protocol tries to anticipate the majority of issues that might emerge in the disclosure process, HIV disclosure is a complex process and there are individual situations that might not be fully addressed within the framework of the NDP.

SECTION II: BACKGROUND

Introduction

As HIV becomes more of a chronic disease and PLHIV live longer, disclosure of HIV status is encouraged; it is viewed as an important way to reduce risky sexual behavior and transmission of the virus. Disclosure can also decrease stigma faced by PLHIV and increase access to support and care (1)(2). In Jamaica, low disclosure rates have been demonstrated among PLHIV linked to care (3). Given that disclosure of HIV status is critical in the control of the spread of HIV (3), the National HIV Programme is focused on providing practical tools to HCP and the wider network of HIV care providers that will facilitate disclosure.

Stigma and discrimination whether real or perceived, continues to pose barriers to the strides that have been made by the National HIV Programme and while many PLHIV are desirous of disclosing their status to partners, family, and friends, they are fearful of the implications of this disclosure. Healthcare Providers who are central to the process of care for PLHIV require tools that will help them to navigate the complex process of HIV status disclosure. The NDP aims to provide a standardized approach to HIV status disclosure and provide the tools required by HCP to anticipate, plan and help PLHIV to overcome barriers and challenges that may emerge as a result of HIV status disclosure.

There is recognition that disclosure of HIV status supports risk reduction and facilitates access to prevention and care services (4). Additional benefits also include increase in trust, increased condom use for HIV prevention and greater access to support (4). These positive effects have an overall impact on PLHIV and their treatment outcomes. Creating a supportive environment that encourages PLHIV who are fearful or experience barriers in disclosing their seropositive status is an essential action that ought to be carried out by HCP.

Definitions

Disclosure: refers to the act of informing any individual or organization (such as a health authority, an employer or a school), of the serostatus of an infected person, or it refers to the fact that such information has been transmitted, by any means, by the

person him or herself, or by a third party, with or without consent. Except in exceptional circumstances, when disclosure to another person is required by law or ethical considerations, the person with HIV has the right to privacy and to exercise informed consent in all decisions about disclosure of his/her status (10).

In simple terms, for the PLHIV, ***disclosure*** is the process of revealing their status or telling someone that he/she is living with HIV (5,6).

Types of Disclosure:

- Full Disclosure: providing full information and knowledge about one's HIV status to the recipient.
- Partial Disclosure: Disclosing information about the presence of a sexually acquired disease without using the name HIV (This can be a step to full disclosure).
- Accidental Disclosure: It usually occurs unintentionally when HIV diagnosis or treatment is being discussed and the recipient overhears; it may also occur when medication is discovered. It involves sharing an HIV status without preparing the recipient for the disclosure, disclosing to them by mistake or inadvertently. .
- Deception: ascribing one's condition to a different chronic illness or linking the PLHIV's behaviour and appearance (e.g., clinic attendance, weight loss); to another chronic condition (such as cancer, diabetes, etc).
- Complete Non-Disclosure: maintaining complete secrecy around diagnosis; not telling the truth or not providing any information about the diagnosis.

Linkage to Care: a client is considered linked to care when he/she has had a medical visit after being confirmed HIV positive (7).

Benefits of Disclosure

There are documented benefits of disclosure for the PLHIV, the HCP and the overall HIV control and prevention programme.

Benefits for the PLHIV

- *Improved psychosocial support, wellbeing, and mental health status.*
- *Enhanced feeling of self-worth, and greater control over one's own health and body.*
- *The feeling of relief, no need to maintain secrecy, which helps to build confidence and resilience.*
- *The ability to more openly access and participate in the decision-making process of one's health care*
- *Improved overall health outcome.*

For the HCP

- *Increased capacity to openly talk about HIV with client and their trusted confidant.*
- *Allows for easier provision of Sexual and Reproductive Health (SRH) education with emphasis on HIV prevention, pre- and post- exposure, prophylaxis, etc.*
- *Reduced barriers to accessing referrals for additional support services.*

For the Healthcare System / Community

- *Greater level of adherence, clinic attendance and active involvement in treatment and care. These result in greater viral suppression, and greater efficiencies in the HIV programme*
- *Helps to reduce stigma and discrimination associated with HIV.*
- *Enhances overall knowledge in the community about sexual reproductive health.*

Challenges and Barriers to Disclosure

The barriers to disclosure are three pronged: those at the level of the individual PLHIV, those at the level of the healthcare system and those at the level of the wider society.

At the level of the PLHIV

- *Insufficient knowledge and understanding about their illness (SRH and treatment literacy). This is also related to the level of educational attainment and ability to process and communicate health information.*
- *Deficient psychosocial support which includes the lack of adequate family support, supportive relationships, peer networks, religious affiliations, etc.*
- *Inadequate financial independence and support which is related to level of education and employment status*
- *Other medical conditions, such as other chronic illnesses, mental health problems to include mood altering drug misuse, etc.*
- *Fear of the possible negative implications of disclosure including the potential of violence, further discrimination, and mental trauma*
- *Self- stigma and difficulties accepting a positive HIV status diagnosis*

At the level of the Family

- *Risk of intimate partner violence*
- *Power dynamics between parents and children, in which parents find it difficult to share sensitive information which may portray them in a vulnerable position.*
- *Rejection from household members which may lead to homelessness*
- *Discomfort and lack of knowledge about how to address sexual and reproductive health issues*

At the level of the Healthcare System

- *Discomfort of HCP about handling disclosure*
- *Inadequately trained healthcare staff to manage the disclosure process*

- *Lack of standardized approach to facilitate HIV status disclosure*
- *Inadequately equipped space to cater for a confidential exchange within the healthcare setting*
- *Lack of confidentiality among HCP*
- *Lack of time and sufficient personnel to facilitate disclosure along with the other activities that they are required to carry out*

At the Societal Level

- *Stigma and discrimination against persons with HIV*
- *Lack of adequate social support programmes for PLHIV*
- *The culture of violence, gender-based violence.*
- *Lack of knowledge and understanding about HIV and SRH issues.*
- *Inadequate redress systems for abuse and breaches of confidentiality.*

Risks of Disclosure

After HIV status disclosure there is the possibility of PLHIV experiencing negative consequences. These include:

- Physical and psychological harm
- Loss of financial or economic support
- Exposure to increased stigma and discrimination
- Loss of social support from family, friends, etc.
- Homelessness

HIV status disclosure could also damage the trust relationship between the HCP and PLHIV and lead to disruption in the care that the PLHIV accesses.

These risks should be discussed with the PLHIV and careful assessment done, and plans put in place to mitigate against these risks. Careful planning with the development of contingencies provides the best chance to reduce these possible negative effects. The plans put in place should identify the most appropriate timing for disclosure and

pinpoint possible healthcare, financial and social support services in the event that these negative risks materialize.

Both the HCP and PLHIV should clearly understand that even with the best planning there is the possibility of unforeseen negative outcomes of HIV status disclosure.

Ethical Considerations Surrounding HIV Disclosure

HIV status disclosure generates a number of ethical, moral and legal issues that require consideration. Healthcare providers have as tenets of their professional practice the obligation to maintain the **confidentiality** of the information shared during their encounters with clients. This is not to be confused with keeping the information a secret, as HCP are obligated to report certain illnesses such as HIV to the public health authorities and also utilize the information within care teams to ensure the best care for their clients.

The PLHIV also has *autonomy* which is the right to make decisions about their own health and future which includes making decisions as to sharing the seropositive status. There is also the principle of *justice*, being fair or just to the wider community protecting them from potential negative actions (11).

HCP may be faced with a dilemma to breach confidentiality when information shared during the patient encounter might help to protect others from contracting an illness or prevent a criminal offense. In the case of potential criminal / legal situations such as:

- When serious harm may occur to a third party, whether or not a criminal offense, e.g. threat of serious harm to a named person
- When HCP believe a patient to be the victim of abuse and the patient is unable to give or withhold consent to disclose
- When, without disclosure, HCP could not act in the overall best interests of a child or young person who is his/ her patient and incapable of consenting to disclosure.
- When, without disclosure, the task of preventing or detecting a serious crime by the police would be prejudiced or delayed (11).

Healthcare providers should report these situations to their supervisors and to the relevant legal authorities for actions to be taken.

The legal requirement for HCP to report positive cases of HIV to the health department should be strictly followed as it ensures that others can be protected, and case investigations can be carried out to detect other individuals who may be undiagnosed.

SECTION III: THE PROCESS OF DISCLOSURE

General Principles of Disclosure

Disclosure is a dynamic life altering process. There are palpable benefits that can be gained for PLHIV and the overall health delivery system. Disclosing one's HIV status to sexual partners, close relatives and friends means talking about sex, sexual orientation, sexual acts, possible mood altering drug misuse, disease, and death (8). All of these are often tabooed topics in Jamaica and are difficult to talk about openly and honestly in most, if not all, societies and communities. Even for the most self-affirming, confident persons, these are difficult subjects to discuss. Not all PLHIV believe that they have a moral obligation to disclose their status to potential partners. Other PLHIV may feel disclosure is unnecessary if they practice safe sex (8). As outlined, there are also negative and undesirable effects that can result from the process and, therefore, proper, assessment, planning, implementation and follow-up which includes the identification of contingencies and social support is required when undertaking HIV status disclosure.

General principles to consider in the process of disclosure:

- The healthcare team and PLHIV should be active participants throughout the process and there should be clear channels of communication established.

- The disclosure process should be flexible, not rushed and should respect the needs of the PLHIV recognizing that these needs may vary at the different stages of disclosure.

- Unless there is an ethical or legal requirement, the PLHIV should not be compelled to disclose their HIV status and HCP should respect the decision of the PLHIV and support their timing.

- The HCP should create the environment in which questions can be freely asked and appropriate responses provided. The environment should also facilitate open

sharing of feelings by both the PLHIV and the party to whom they wish to disclose.

- The HCP should ensure the provision of adequate educational material to the parties involved in the disclosure and should also assess the assimilation of the information provided.

- The HCP should provide access to the network of additional clinical and social support throughout the process. This may require referrals to other services and groups.

- As is necessary and when considered to be in the best interest of the client, peer navigators and other PLHIV support groups should be integrated in the process of disclosure.

- PLHIV should be informed of the benefits of HIV status disclosure and encouraged to share their positive status where the benefits of disclosing outweigh the potential risks of negative outcomes from such disclosure, as well as where their status disclosure might lead to reducing the risk of transmission, or enable other individuals to access HIV treatment care and support services.

When should the disclosure process begin?

The disclosure process should begin once the PLHIV is linked to care.
Further steps in the disclosure process will depend on the PLHIV's level of acceptance of their diagnosis and the proceeding special considerations in relation to the individual case and the readiness of both the client and the recipient.

Special Considerations for Disclosure

There are some groups of PLHIV that are at increased risk of a negative disclosure outcome. These groups include;

- ✔ Members of the LGBTQ community who are PLHIV faced with additional stigma and discrimination based on their sexual orientation and this should be taken into consideration when assessing the timing as well as the appropriateness of the disclosure to targeted recipients of the disclosed information.

- ✔ Persons living with disabilities should be assessed to ascertain the most appropriate tactics and a team-based approach including members of the psychosocial support team.

- ✔ PLHIV who are in abusive relationships or at risk of intimate partner abuse should not be encouraged to disclose if it is assessed that the disclosure may expose them to further abuse.

- ✔ PLHIV with coexisting chronic medical conditions as disclosure should be facilitated when these clients are at optimal clinical and psychological states. Ideally, chronic medical conditions (mental health disorders, addiction disorders, diabetes, hypertension, etc.) should be under control except in cases where the HIV status disclosure itself might lead to improved psychosocial support, improved wellbeing, better mental health status and increased access to care that might lead to the control of these chronic conditions.

- ✔ In adolescents under the age of 18, disclosure conversations facilitated by HCP should involve the participation of parents and/or guardians and, should be carried out in accordance with the national health guidelines for HIV disclosure in children and adolescents.

- ✔ Also, for adolescents under age 18, the HCP should refer to the national guidelines for HIV disclosure in children and adolescents to ensure appropriate assessment, preparation and age-appropriate communication material and techniques are utilized.

- ✔ For pregnant mothers, if there are adverse effects of HIV status disclosure, this could lead to reduced maternal psychological and physical wellbeing resulting in poor prenatal growth and fetal development as well adverse pregnancy outcomes.

Stages of Disclosure

The disclosure process is dynamic and should be carried out by members of the care team namely, doctors, nurses, psychologist, social workers, case managers, contact investigators, adherence counsellors. Apart from the clear objectives and steps, there should be contingencies in the event that the process does not go according to plan or in case the PLHIV decides to abandon the process at any point.

The care offered to PLHIV is a team effort and the member of the team who enjoys the best rapport with the PLHIV should ideally be the individual who leads the process of disclosure for that particular client.

The lead HCP and other members of the care team ought to be trained in the process of disclosure prior to initiating the process.

Preparation for the HCP

The HCP should:

- ✔ Receive competence-based training in process of disclosure and should have adequate knowledge of HIV and other SRH matters.
- ✔ Be aware of and have access to wider support services that may be required before, during and after disclosure.
- ✔ Develop a strong trust relationship with the PLHIV. It is important to utilize a team approach involving other appropriate members of the healthcare team as may be required.
- ✔ Build rapport and establish appropriate levels of respect and confidentiality.
- ✔ Ensure appropriate detailed documentation in the healthcare records of the dates

and activities carried out in support of the disclosure process.

It is important that clear documentation including the dates and any additional comments are made in the patient record so as to facilitate continuity of care.

The stages of disclosure are;

1. Assessment
2. Planning
3. Implementation
4. Follow-Up

Stage 1: Assessment

During this stage, the PLHIV readiness for disclosure as well as the recipient's readiness (appropriateness) for receiving the information is assessed.

The assessment should be carried out by a member of the care team (doctor, nurse, psychologist, social worker, case managers, contact investigator, adherence counsellor). The confidentiality of the PLHIV should be maintained and the assessment should be carried out in a setting suitable for counseling.

The assessment should not be rushed, adequate time ought to be dedicated to exploring the views of the PLHIV.

The following tool is utilized to do the assessment, and additional comments should be made to highlight relevant issues that may not be captured by the questions contained in the tool.

PLHIV Readiness for Disclosure Assessment

HCP should respond to the following questions based on discussions with PLHIV:		
1	Has the PLHIV accepted their seropositive diagnosis of HIV?	Yes No
2	Does the PLHIV have adequate knowledge about HIV?	Yes No
3	Is the PLHIV desirous of disclosing their seropositive status?	Yes No

4	a. Are all other medical condition(s) of the PLHIV under control? b. Will disclosure result in better control of the PLHIV's other medical condition(s)?	Yes Yes	No No	N/A N/A
5	**If** PLHIV has any of the following special circumstances that increase vulnerability (LGBTQ, adolescent, chronic physical or mental illness, physical disability, in an abusive relationship, pregnant mother), has it been adequately discussed?	Yes	No	N/A
6	Does the PLHIV have economic independence / adequate socioeconomic support?	Yes	No	
PLHIV who obtain a YES to all of the questions should be considered for disclosure planning, implementation and follow up.				
For PLHIV who have obtained NO to any of the above questions, the HCP should continue to work with the PLHIV and re-evaluate in subsequent healthcare visits (1-3 months).				

Recipient Readiness Assessment

HCP should obtain responses from the PLHIV on the following questions:			
1	Is the recipient knowledgeable about HIV?	Yes	No
2	Has the recipient expressed positive views about PLHIV?	Yes	No
3	Does the PLHIV have a functional relationship with the recipient?	Yes	No
4	Has the recipient demonstrated the capacity to keep information confidential?	Yes	No
5	Has the recipient expressed positive attitudes towards vulnerable groups?	Yes	No
PLHIV who answered YES to all of the questions regarding the recipient should be considered for disclosure implementation within 1 month.			
If the response to any of the above questions is NO then the recipient is not likely a good candidate to receive disclosure information. HCP should work with the recipient to increase knowledge about HIV and revaluate in 1-3 months.			

Stage 2: Planning

For disclosure to be successful, there needs to be adequate planning and preparation of the PLHIV and the Recipient by the members of the healthcare team. The planning for disclosure involves:

a) Preparation of the PLHIV

b) Preparation of the Recipient

a. Preparation of the PLHIV

Based on the assessment of the PLHIV, steps should be taken by the HCP to address the deficiencies identified. The HCP should initiate steps to improve the level of preparedness of the PLHIV by addressing:

- ✔ The PLHIV's level of acceptance, understanding of their diagnosis, treatment, and care for HIV (diagnosis and treatment literacy)
- ✔ The PLHIV's understanding about the implications, risks and potential benefits for disclosing their seropositive status.
- ✔ The PLHIV's special circumstances such as increased vulnerability due to LGBTQ, disability, mental illness, etc.
- ✔ The timing for disclosure and identifying potential social and economic support that may be required.
- ✔ The PLHIV's reasons for disclosing and understanding the relationship between the PLHIV and the recipient of the disclosure information.

The preparation for PLHIV may be a lengthy process that lasts over several months as many PLHIV have complex issues that require multiple levels of interventions to address, which are not always totally dependent on the HCP and the health services. The target should be to carry this out within 1-3 months. The HCP should be satisfied that the interventions have been carried out and that they meet the areas outlined in the planning tool for PLHIV.

PLANNING PLHIV PREPARATION

HCP should answer the following questions about interventions carried out in favor of the PLHIV:			
1	Has there been an accepted response by the PLHIV about seropositive diagnosis of HIV?	Yes	No

2	Is there a greater understanding of the risks, barriers and benefits of disclosure?	Yes No
3	Have there been special interventions that help to address the vulnerabilities of the PLHIV?	Yes No N/A
4	Have there been interventions to address the social issues/ relationship issues affecting the PLHIV?	Yes No N/A
If the response is NO to any of the above questions, the HCP should continue to work with the PLHIV and re-evaluate in subsequent healthcare visits (1-3 months).		

b. Preparation of the Recipient

The recipient of the HIV status disclosure in the majority of cases is identified by the PLHIV. The exception to this is when the HCP shares the information because of a legal requirement . The recipient identified by the PLHIV is usually a sibling, child, parent/ guardian, sexual partner, friend, or caregiver.

Before interacting with the identified recipient, it is important to carry out the recipient assessment so as to identify the potential issues that need to be the focus of the HCP interventions as well as determine if the recipient is a good candidate to receive the information. The initial assessment about the recipient should be gathered from the PLHIV. However, prior to continuing the disclosure steps with a potential recipient, the HCP should also carry out the assessment and inform the PLHIV of the results and together decide on whether to continue with the identified recipient or abandon the process and possibly select another potential recipient for HIV status disclosure.

The areas to focus include:

- The reasons for disclosing to the recipient identified.
- The knowledge and understanding of the recipient about HIV (transmission, clinical progression, treatment, prevention etc.)
- Whether the recipient has a sufficiently close and healthy relationship with the PLHIV.
- The ability of the recipient to maintain the confidentiality of the PLHIV status, this can be explored by examining how the individual has dealt with confidential information in the past.
- History of discriminatory behaviour and other negative reactions towards PLHIV
- History of discriminatory or negative behavior towards persons with disabilities,

illnesses, and sexual minorities such as LGBTQ, etc.

Preparing the recipient requires a special set of skills and procedures so that the disclosure does not take place inadvertently and prematurely. The HCP should keep the PLHIV informed about the progress of conversations with the recipient. The process may require multiple sessions and should be completed over 1-3 months.

PLANNING RECIPIENT PREPARATION

HCP should answer the following questions about interventions carried out in favor of the Recipient:				
1	Has there been an accepted response by the Recipient about a seropositive diagnosis of HIV (signs, symptoms, transmission, disease progression, etc.?	Yes	No	
2	Has the Recipient demonstrated the ability to maintain confidentiality after the HCP interventions?	Yes	No	
3	Has the Recipient demonstrated acceptable non-discriminatory responses to PLHIVs and other vulnerable groups (LBGTQ, disabilities, etc.)	Yes	No	
4	Have there been acceptable responses to the interventions to address the reasons for disclosing / relationship issues between the PLHIV and the Recipient?	Yes	No	N/A
If the response is NO to any of the above questions, the HCP should continue to work with the Recipient and re-evaluate in subsequent healthcare visits (1-3 months).				

Stage 3: Implementation

The PLHIV who have been assessed and prepared for disclosure with a prepared recipient and HCP should progress to the stage of implementation. The implementation stage will look at the type of disclosure, timing, setting and support.

Type of Disclosure

The HCP should provide information on the options for disclosure and the decision on the type of disclosure should ultimately be made by the PLHIV. The PLHIV should be encouraged to fully disclose and where partial disclosure is utilized it should be done as a step to full disclosure. The HCP should not be actively engaged in deception and PLHIV should be given relevant tips to avoid accidental disclosure.

Timing

Adequate timing is difficult to assess; however, the following questions should be explored by the HCP:

1	Is there any current emotionally challenging situation for the PLHIV / Recipient? a. Income loss b. Social situation c. Relationship challenges d. Housing challenges	Yes No Yes No Yes No Yes No Yes No
Issues to be considered include economic or social trauma such as loss of income, death or illness of loved one, divorce, relocations, loss of assets, etc.		

Setting

The HCP and the PLHIV should decide on the setting for disclosure. This can be done in the healthcare, home or other setting with or without the presence of the HCP. The HCP should offer to be present.

2	Where would the PLHIV like the disclosure to take place?	Healthcare setting Home setting Other
3	Is the PLHIV desirous of the HCP to be present at the disclosure?	Yes No
When disclosure is to occur within the healthcare setting with the participation of HCP, a period of at least 1-2 hours should be made available for the disclosure to take place.		

Support

The HCP should review, with the use of role play, the information that will be disclosed to the recipient. Post disclosure, an assessment should be made of how the recipient has taken the information shared, and plans be made for possible negative repercussions. The following factors should be considered:

4	Has the HCP participated in role play with PLHIV surrounding disclosure?	Yes No
5	Does the PLHIV have a plan should there be a negative reaction to the disclosure?	Yes No

6	Has the HCP identified support services that may be accessed by the PLHIV in the case of a negative reaction?	Yes No
If the response is YES to all of the above questions, disclosure may be carried out within 1 month.		
If the response to any of the above questions is NO then the HCP should continue to work with the PLHIV to increase preparedness to disclose and revaluate in 1-3 months.		
The PLHIV plan for a negative reaction should include possible economic and social support as may be required. A note should be made in the medical records of the proposed time, date as well as the recipient's name and contact information.		

Stage 4: Follow-Up

The HCP should make early contact with the PLHIV to ascertain the outcome of the disclosure (within 1-3 days) and, subsequently, the PLHIV should be evaluated on each healthcare visit via the following questions:			
1	Has the recipient expressed any negative reactions to the disclosure?	Yes	No
2	Were there any unanswered questions from the disclosure?	Yes	No
	If Yes, what were those questions? State and list recommended action.		
3	Does the PLHIV / Recipient require additional support?	Yes	No
3a	If Yes, what additional support is required?		
	i. HIV/ SRH Education	Yes	No
	ii. Counselling sessions	Yes	No
	iii. Referral to other support services	Yes	No
	iv. Other	Yes	No
	Documentation should be made in the medical records of any unresolved issues, actions, referrals and additional support required.		

Evaluation of the Stages of Disclosure

It is important that the HCP systemically evaluates the progress made under the four stages of disclosure and adjust based on gaps that may be identified.

All disclosure activities carried out should be documented and placed in the client's medical records. Where specific disclosure tools are utilized care should be taken to ensure that the patient information is included on the tools and that they are incorporated into the patient clinic record.

In Stage 1 Assessment: The HCP should identify if the PLHIV/ Recipient is ready for

disclosure yes or no. This evaluation should be carried out by the HCP who is leading in the disclosure process with the PLHIV and ought to take place when the PLHIV is linked to care. If either the recipient or the PLHIV is not ready then the interventions which include; education, the provision of psychosocial support etc. should be instituted, and documented and a further assessment carried out within 1-3 months.

In Stage 2 Planning: The evaluation of the planning stage involves assessing preparation of the PLHIV and the Recipient. This should be done by the team leader and documentation made in the patient's record when the PLHIV and Recipient were prepared. Comments should be made to indicate the status of the attainment of the objectives of the planning stage. This stage should be reviewed and signed off by at least two members of the care team (e.g. Social Worker & Psychologist) and, if adequate, there should be progression to Stage 3. If not, then adjustments should be made, and a review should be done in 1-3months.

In Stage 3 Implementation: The implementation tool should be utilized to determine the timing, setting and the support for implementation required. The outcome of the assessment should be documented and if deemed to be adequate by the HCP the implementation should be carried out and the patient progressed to the subsequent stage. If not the HCP should work with the PLHIV for an additional 1-3 months and then be reassessed.

In Stage 4 Follow-Up: At this stage the HCP is to document efforts made after the disclosure has occurred to evaluate the reaction and or need for further interventions by the healthcare and support teams. The Follow-Up tool captures the specific parameters that should be explored within 1-3 days and subsequently at each visit with the PLHIV.

SECTION IV: DISCLOSURE SUPPORT TOOLS

a. Individualized Disclosure Plan

The healthcare team should include disclosure within the context of the general services provided to the PLHIV who have been linked to care and develop an *individualized disclosure plan*. This is defined as a step-by-step approach to the disclosure process which includes milestones established with the HCP and the PLHIV. The plan should contain the disclosure stages and timelines for their attainment.

The timelines included in the plan should be used as a guide; they should be flexible to accommodate changes in circumstances affecting the PLHIV throughout the progressions of the disclosure stages.

This section contains the Individualized Disclosure Plan, which contains the relevant tools to undergo the various stages of disclosure. The Plan should be completed by the HCP. The HCP should circle the responses provided by the PLHIV and, where required, concise comments should be made. The tool should be placed in the medical records of the PLHIV. Each page of the tool should have the patient's name and medical record number.
For persons who have undergone the Implementation and are in the Follow-Up stage, only the Follow-Up portion of the Plan should be completed by the HCP on subsequent visits.

b. The main messages that need to be discussed during partial and full disclosure.

It is important that the HCP is reminded of the main messages that should be reinforced during the partial and full disclosure process (8). These messages help the PLHIV and Recipients of the disclosure information to develop positive attitudes and become more knowledgeable about HIV. In the longer term, skills are developed that facilitate positive living and have the potential to decrease overall societal stigma. The skills should also

enable better access to the healthcare services.

c. Disclosure Case Scenarios

The case scenarios are of typical situations that might be encountered by HCP as they explore disclosure with their clients. The scenarios identify areas of focus for interventions by HCP at each stage of the disclosure process.

d. Sample Completed Forms

An outline of a sample of how the forms would be completed for a typical case of disclosure.

e. The Use of Role Plays

The definition, benefits and general information is provided about the use of role plays in the disclosure process.

f. Algorithm for the Disclosure Process in Adult PLHIV

The algorithm for the disclosure process is a job aid which summarizes the main steps, timelines and activities.

INDIVIDUALIZED DISCLOSURE PLAN

Health Facility _____ **HCP** _____
Patient _____ **Case number** _____
Date _____ **Time** _____

ASSESSMENT

PLHIV Readiness for Disclosure Assessment

HCP should respond to the following questions based on discussions with PLHIV:

1	Has the PLHIV accepted their seropositive diagnosis of HIV?	Yes No
2	Does the PLHIV have adequate knowledge about HIV?	Yes No
3	Is the PLHIV desirous of disclosing their seropositive status?	Yes No
4	a. Are **all other** medical condition(s) of the PLHIV under control? b. Will disclosure result in better control of the PLHIV's other medical condition(s)?	Yes No Yes No
	Name of condition(s)	Under control?
a.		Yes No
b.		Yes No
c.		Yes No
d.		Yes No
e.		Yes No
Comments:		
5	**If** PLHIV has any of the following special circumstances that increase vulnerability, has it been adequately discussed? **a.** LGBTQ+ **b.** adolescent (10-15 years) **c.** chronic physical or mental illness **d.** physical disability **e.** an abusive relationship **f.** pregnant mother **g.** drug misuse	 Yes No N/A Yes No N/A Yes No N/A Yes No N/A Yes No N/A Yes No N/A Yes No N/A
6	Does the PLHIV have economic independence / adequate socioeconomic support?	Yes No

PLHIV who answered **YES to all** of the questions should be considered for disclosure planning within 1-3 months.
For PLHIV who have responded **NO to any** of the above questions, the HCP should continue to work with the PLHIV and revaluate in subsequent healthcare visits (1-3 months).

Recipient Readiness Assessment

HCP should carry out the assessment by obtaining responses from the PLHIV or via discussions with the potential recipient on the following questions:

1	Do you think _____ (recipient) is knowledgeable about HIV, for e.g. how it is spread, impact on one's health, etc.?	Yes	No
2	Has _____(recipient) expressed positive views about PLHIV?	Yes	No
3	Is there currently a meaningful relationship between the PLHIV and the recipient?	Yes	No
4	Is _____ (recipient) able to keep information private and confidential?	Yes	No
5	Has _____ (recipient) expressed positive attitudes towards vulnerable groups, for e.g. persons with disabilities, illnesses, LGBTQ, pregnant mothers, etc.?	Yes	No

If the answer is **YES to all** of the questions, the recipient should be considered for disclosure planning within 1-3 months.

If the response is **NO to any** of the above questions, then the recipient is not likely a good candidate to receive disclosure information. HCP should continue to work with the PLHIV / potential recipient to increase knowledge about HIV and revaluate in 1-3 months.

Comments:

HCP signature _____ Date completed_____

PLANNING

PLHIV PREPARATION

HCP should answer the following questions about interventions carried out in favor of the PLHIV:

1	Has there been an accepted response by the PLHIV about seropositive diagnosis of HIV?	Yes No
2	Is there a greater understanding of the risks, barriers and benefits of disclosure?	Yes No
3	Have there been special interventions that help to address the vulnerabilities of the PLHIV?	Yes No N/A
4	Have there been interventions to address the social issues/ relationship issues affecting the PLHIV?	Yes No N/A

If the response is NO to any of the above questions, the HCP should continue to work with the PLHIV and re-evaluate in subsequent healthcare visits (1-3 months).

Comments:

HCP signature _____ Date completed_____

RECIPIENT PREPARATION

HCP should answer the following questions about interventions carried out in favor of the Recipient:

1	Has there been an accepted response by the Recipient about a seropositive diagnosis of HIV (signs, symptoms, transmission, disease progression, etc.?	Yes No
2	Has the Recipient demonstrated the ability to maintain confidentiality after the HCP interventions?	Yes No
3	Has the Recipient demonstrated acceptable no discriminatory response to PLHIVs and other vulnerable groups (LBGTQ, disabilities, etc.)	Yes No
4	Have there been acceptable response to the interventions to address the reasons for disclosing / relationship issues between the PLHIV and the Recipient?	Yes No N/A

If the response is NO to any of the above questions, the HCP should continue to work with the Recipient and re-evaluate in subsequent healthcare visits (1-3 months).

Comments:

HCP signature _____ Date completed_____

IMPLEMENTATION

HCP should ask PLHIV the following:

1.	What type of disclosure would you like to carry out? a. Full Disclosure b. Partial Disclosure (only used as a step to Full Disclosure)	 Yes No Yes No
2.	Are you **or** the recipient currently facing an emotionally challenging situation? a. Income loss b. Social situation c. Relationship challenges d. Housing challenges e. other (e.g. loss of income, death or illness of loved one, divorce, relocations, etc)	Yes No Yes No Yes No Yes No Yes No Yes No
Briefly describe situation:		

Setting

HCP should ask PLHIV the following:

2	Where would you like the disclosure to take place?	Healthcare setting Home setting Other	[ ] [ ] [ ]
3	Would you like a HCP to be present at the disclosure?		Yes No
When disclosure is to occur within the healthcare setting with the participation of HCP a period of at least 1-2 hours should be made available for the disclosure to take place.			

Support

HCP should answer the following:

4	Has the HCP participated in role play with PLHIV surrounding disclosure?	Yes No
5	Is there a plan should there be a negative reaction to the disclosure?	Yes No
6	Have support services that may be accessed by the PLHIV in the case of a negative reaction been identified by HCP?	Yes No
7	Does the PLHIV have a clear understanding of the risks and potential implications of the disclosure?	Yes No

If the response is **YES to all** of the above questions, disclosure may be carried out within 1 month.

If the response to **NO to any** of the above questions, then the HCP should continue to work with the PLHIV to increase preparedness to disclose and revaluate in 1-3 months.

The PLHIV plan for a negative reaction should include possible economic and social support as may be required. A note should be made in the medical records of the proposed time, date as well as the recipient name, relation and contact information.
Comments: HCP signature _____ Date completed_____

FOLLOW UP

Early Follow up (within 1-3 days) after disclosure

Patient Name:_____ Recipient Name: _____Relation_____ Contact #_____

1	Did _____ (recipient name) express any negative reactions to the disclosure?	Yes	No
2	Were there any unanswered questions from the disclosure?	Yes	No
If Yes, what were those questions? State and list recommended action. Question 1: Action: Question 2: Action:			
3	Does the PLHIV / Recipient require additional support?	Yes	No
3a	If Yes, what additional support is required?		
	i. HIV/ SRH Education	Yes	No
	ii. Counselling sessions	Yes	No
	iii. **Other** (state name of the service):: _____	Yes	No
Comments: HCP signature _____ Date completed_____			

Later/Routine Follow up (1-6 months) after disclosure.

Patient Name: _____Recipient Name: _____
Relation_____Contact #_____

1	Has _____ (recipient name) expressed any negative reactions to the disclosure?	Yes No
a. If Yes, briefly describe the effects of the negative reaction(s):		
b. If Yes, list recommended actions: **Action 1:** **Action 2:**		
2	Does the PLHIV / Recipient require additional support?	Yes No
a. **If Yes, what additional support is required?** i. **HIV/ SRH Education** ii. **Counselling sessions** b. **Other** (state name of the service)**:** _____		Yes No Yes No Yes No
Comments: HCP signature _____ Date completed_____		

The main messages that need to be discussed during partial and full disclosure.

Main Messages for Partial Disclosure:

- Explain the principles of the immune system working to protect an organism from infections.
- Identify the symptoms (current and past) if any can be linked to the disease.
- Explain the ability to be living with a chronic disease and not feeling sick.
- Explain the importance of taking treatment and their positive effect on microbes and viruses.
- Explain recovery from being sick or preventing becoming sick as related to taking medicines.
- Identify the medicines that are being taken (e.g., by shapes and sizes)
- Explain the need for regular check-ups and living a healthy lifestyle.
- Explain the importance of sharing information with the caregiver and provider.

Main Messages for Full Disclosure (In addition to the partial disclosure messages):

- Name HIV and explain modes of transmission.
- Address stigma and confidentiality
- Counsel on empowerment for living positively (8).

Disclosure Case Scenarios

Case I

Marva is a 30-year-old female who has 2 children, 5-year-old Daniel and 3-year-old Brittany. Marva is now 4 months pregnant and during the routine blood works for her antenatal care she was diagnosed as having HIV. Both of her children were tested and are HIV negative. Marva was placed on ART and is currently virally suppressed. Marva lives with Donavan who is the father for Britany and the unborn child. Marva and Donavan have been together for 4 years and they have a relationship that Marva describes as “ok” she says they have fought in the past to the point that she had to report him to the police, but she says they haven’t had any recent issues. Donavan is a farmer and Marva works in a bar from time to time. She says Donavan supports her and the 2 children but sometimes has a bad temper. In her last visit with the psychologist at the treatment site Marva expressed a desire to disclose to Donavan her positive HIV Status as she wants him to get tested and if positive to be started on treatment

HCP should focus on:

Special Considerations: *pregnancy, previous history of intimate partner violence, inconsistent income*

Risk of Disclosure: *physical and psychological abuse, loss of financial support, effects on unborn child*

Benefits of Disclosure*: Psychosocial support, increased adherence, partner accesses screening, prevention and treatment services.*

Assessment*: Marva’s level of acceptance of her diagnosis, Donavan’s knowledge about HIV, ability to manage confidential information and behavior toward PLHIV and other vulnerable groups.*

Planning*: Discuss with Marva resources available in case of negative disclose, contingency planning for support: financial, housing for Marva and her children etc. Evaluate and improve Donavan’s knowledge about HIV, evaluate his ability to deal with information about HIV diagnosis as well as his views on PLHIV. Evaluate his likely*

behavior towards the news that Marva has HIV and that he and his unborn child might be exposed. HCP should be careful not to inadvertently disclose Marva's status. Identify support groups and entities that may provide support to Marva

Implementation: *Discuss with Marva the date, location at home or at the health facility and presence of the HCP*

Follow Up: *Identify if Donavan has expressed any negative behavior about Marva's HIV status. Provide Marva, or Donovan any additional support or information. Ensure Donavan accesses screening, prevention and treatment services as may be required.*

Case II

Zavia is a 20-year-old male who is currently enrolled at university in the second year of a 3-year course pursuing his bachelor's degree in education. Zavia is supported by his mother and father who are devout members of a conservative religious group in fact his father Charles, leads a congregation. He also has a younger sister Zhara who is 18 years old. At a recent health fair Zavia did a HIV test and got a positive result. He later went to the health centre and got a confirmative test and after 2 months he decided to commence ART therapy. Though Zavia has a very close relationship with his parents and sister, they do not know that he is gay and that he has been engaging in sexual activity since the age of 16. He would like to be able to share his status with his family, but he is concerned that he might be banished from his family and that he might lose the financial and educational support that they currently provide.

HCP should focus on:

Special Considerations: *LGBTQ male*

Risk of Disclosure: *psychological abuse, loss of financial support, stigma and discrimination about sexuality, loss of housing*

Benefits of Disclosure*: Psychosocial support, increased adherence.*

Assessment*: Zavia's level of acceptance of his diagnosis, Recipient's knowledge about HIV, ability to manage confidential information and behavior toward PLHIV and gays.*

***Planning**: Discuss with Zavia resources available in case of negative disclosure, contingency planning for support: financial, housing, school etc. Discuss with Zavia the member(s) of his family that might be best able to handle the information. Evaluate and improve the identified recipient's knowledge about HIV, evaluate his/her ability to deal with information about HIV diagnosis as well as their views on PLHIV. Identify support groups that may offer assistance to Zavia. Evaluate the recipient's likely behavior towards Zavia regarding the news that he has HIV and that he is gay.*

***Implementation:** Discuss with Zavia the date, location, at home, or at the health facility, and presence of the HCP. The timing of the disclosure should be discussed with Zavia and considerations made whether or not to delay the disclosure.*

***Follow Up:** Identify if Zavia has experienced any negative behaviour from his family member(s) about his HIV status and provide Zavia and the recipient with any additional support or information.*

Case III

Roy is a 55-year-old male who has been living with HIV for over 4 years, Roy also has diabetes for over 6 years which is poorly controlled, as he doesn't take his diabetes medication consistently. Roy has been on and off his ART medication since his diagnosis, however, for the last 2 months he reports that he has been consistently taking his HIV medicines. Roy has not disclosed his HIV status to anyone as he says it's nobody's business and that he is an independent man, and he can take care of himself. Roy has 2 Children, John aged 32 years and Marcia age 27. Roy over the past 3 months has had progressive loss of his vision and is unable to see. He is unable to go anywhere unassisted and he can barely read to take his medications and to carry out his regular daily activities. The psychologist and social worker at the clinic where Roy goes for care have been talking to Roy over the last 3 months to encourage him to disclose his status to one of his children who are the ones who take him to the clinic for his appointments. Even though Roy is close his daughter Marcia he has so far resisted the efforts of the health team as he doesn't want anyone to know that he is HIV positive.

HCP should focus on:

Special Considerations: *Roy is visually impaired, has poorly controlled diabetes and has poor history of diabetes medication and ART compliance.*
Risk of Disclosure: *potential HIV related stigma & discrimination*
Benefits of Disclosure*: Psychosocial support, increased adherence possible better control of diabetes and HIV. Provision of assistance to access care services.*
Assessment*: Roy does not desire disclosing his status, HCP has to work with him to enhance his knowledge about the benefits of disclosure to a recipient of his choice. Explore Roy's openness to the possibility of partial disclosure as a step towards full disclosure and the potential recipient's knowledge of HIV and ability to manage confidential information.*
Planning*: If Roy progresses to this stage then discuss with him possible negative reactions and contingencies for support from other family members/ friends.*
Implementation: *Discuss with Roy the date, location at home or at the health facility and presence of the HCP.*
Follow Up: *Identify if Roy has experienced any negative behaviour from his family member about his HIV status and provide Roy and the recipient with any additional support or information that may be required.*

SAMPLE COMPLETED FORMS CASE II

INDIVIDUALIZED DISCLOSURE PLAN

Health Facility: Comprehensive Health Centre **HCP:** SW Davis
Patient: Zavia Beds **Case number** DII
Date Dec 7, 2025 **Time:** 10:05am

ASSESSMENT

PLHIV Readiness for Disclosure Assessment

HCP should respond to the following questions based on discussions with PLHIV:

1	Has the PLHIV accepted their seropositive diagnosis of HIV?	(Yes) No
2	Does the PLHIV have adequate knowledge about HIV?	(Yes) No
3	Is the PLHIV desirous of disclosing their seropositive status?	(Yes) No
4	c. Are **all other** medical condition(s) of the PLHIV under control?	(Yes) No
	d. Will disclosure result in better control of the PLHIV's other medical condition(s)?	Yes No (N/A)
	Name of condition(s) N/A	Under control?
a.		Yes No
b.		Yes No
c.		Yes No
d.		Yes No
e.		Yes No
Comments: Zavia has no other medical conditions currently		
5	**If** PLHIV has any of the following special circumstances that increase vulnerability, has it been adequately discussed?	
	h. LGBTQ+	(Yes) No N/A
	i. adolescent (10-15 years)	Yes No (N/A)
	j. chronic physical or mental illness	Yes No (N/A)
	k. physical disability	Yes No (N/A)
	l. an abusive relationship	Yes No (N/A)
	m. pregnant mother	Yes No (N/A)
	n. drug misuse	Yes No (N/A)

6	Does the PLHIV have economic independence / adequate socioeconomic support?	Yes (No)

PLHIV who answered **YES to all** of the questions should be considered for disclosure planning within 1-3 months.

For PLHIV who have responded **NO to any** of the above questions, the HCP should continue to work with the PLHIV and revaluate in subsequent healthcare visits (1-3 months).

SW Davis will discuss with Zavia possible financial support and review in 3 months, Zavia has been introduced to a JN+ support group they have been helping him cope with his sexuality and HIV diagnosis. He has also been referred to participate in a creative skills training course carried out by JN+

After 3 months Zavia has gotten a job that covers his expenses, and he is now living in a shared apartment off campus

Recipient Readiness Assessment Zavia Has identified his sister Zhara as the persons to whom he wants to disclose.

HCP should carry out the assessment by obtaining responses from the PLHIV or via discussions with the potential recipient on the following questions:

1	Do you think ____Zhara_Beds_____ (recipient) is knowledgeable about HIV, for e.g. how it is spread, impact on one's health, etc.?	(Yes) No
2	Has _____Zhara_Beds__(recipient) expressed positive views about PLHIV?	(Yes) No
3	Is there currently a meaningful relationship between the PLHIV and the recipient?	(Yes) No
4	Is _____Zhara_Beds____ (recipient) able to keep information private and confidential?	(Yes) No
5	Has _Zhara_Beds_____ (recipient) expressed positive attitudes towards vulnerable groups, for e.g. persons with disabilities, illnesses, LGBTQ, pregnant mothers, etc.?	(Yes) No

If the answer is **YES to all** of the questions, the recipient should be considered for disclosure planning within 1-3 months.

If the response is **NO to any** of the above questions, then the recipient is not likely a good candidate to receive disclosure information. HCP should continue to work with the PLHIV / potential recipient to increase knowledge about HIV and revaluate in 1-3 months.

Comments:
Zhara has expressed positive attitudes and we plan to disclose within 1 month at the clinic.

HCP signature: _SW DAVIS_____ Date completed: March 8, 2026_____

PLANNING

PLHIV PREPARATION

HCP should answer the following questions about interventions carried out in favor of the PLHIV:

1	Has there been an accepted response by the PLHIV about seropositive diagnosis of HIV?	(Yes) No
2	Is there a greater understanding of the risks, barriers and benefits of disclosure?	(Yes) No
3	Have there been special interventions that help to address the vulnerabilities of the PLHIV?	(Yes) No N/A
4	Have there been interventions to address the social issues/ relationship issues affecting the PLHIV?	(Yes) No N/A

If the response is NO to any of the above questions, the HCP should continue to work with the PLHIV and re-evaluate in subsequent healthcare visits (1-3 months).

Comments: Zavia has expressed positive attitudes and is ready to disclose his status to his sister

HCP signature SW DAVIS Date completed: March 8, 2026

RECIPIENT PREPARATION

HCP should answer the following questions about interventions carried out in favor of the Recipient:

1	Has there been an accepted response by the Recipient about a seropositive diagnosis of HIV (signs, symptoms, transmission, disease progression, etc.?	(Yes) No
2	Has the Recipient demonstrated the ability to maintain confidentiality after the HCP interventions?	(Yes) No
3	Has the Recipient demonstrated acceptable no discriminatory response to PLHIVs and other vulnerable groups (LBGTQ, disabilities, etc.)	(Yes) No
4	Have there been acceptable response to the interventions to address the reasons for disclosing / relationship issues between the PLHIV and the Recipient?	(Yes) No N/A

If the response is NO to any of the above questions, the HCP should continue to work with the Recipient and re-evaluate in subsequent healthcare visits (1-3 months).

Comments:
Zhara is a good candidate to handle HIV disclosure information she is knowledgeable about HIV and has not demonstrated any negative attitudes towards PLHIV and LGBT persons.

HCP signature SW Davis Date completed: March 10, 2026

IMPLEMENTATION

HCP should ask PLHIV the following:

1.	What type of disclosure would you like to carry out? a. Full Disclosure b. Partial Disclosure (only used as a step to Full Disclosure)	 Yes No Yes No
2.	Are you **or** the recipient currently facing an emotionally challenging situation? a. Income loss b. Social situation c. Relationship challenges d. Housing challenges e. other (e.g. loss of income, death or illness of loved one, divorce, relocations, etc)	Yes No Yes No Yes No Yes No Yes No Yes No
Briefly describe situation:		

Setting

HCP should ask PLHIV the following:

2	Where would you like the disclosure to take place?	Healthcare setting [X] Home setting [ ] Other [ ]
3	Would you like a HCP to be present at the disclosure?	Yes No
When disclosure is to occur within the healthcare setting with the participation of HCP a period of at least 1-2 hours should be made available for the disclosure to take place.		

Support

HCP should answer the following:

4	Has the HCP participated in role play with PLHIV surrounding disclosure?	Yes No
5	Is there a plan should there be a negative reaction to the disclosure?	Yes No
6	Have support services that may be accessed by the PLHIV in the case of a negative reaction been identified by HCP?	Yes No
7	Does the PLHIV have a clear understanding of the risks and potential implications of the disclosure?	Yes No

If the response is **YES to all** of the above questions, disclosure may be carried out within 1 month.

If the response to **NO to any** of the above questions, then the HCP should continue to work with the PLHIV to increase preparedness to disclose and revaluate in 1-3 months.

The PLHIV plan for a negative reaction should include possible economic and social support as may be required. A note should be made in the medical records of the proposed time, date as well as the recipient name, relation and contact information.

Comments: Zavia wants to disclose on March 16, 2026 at 10 am at the health centre, If there is a negative result he plans to continue attending his support group and he has sufficient financial resources to take care of himself. SW Davis has also arranged for him to get additional counselling sessions with the treatment site psychologist if required.

HCP signature SW Davis Date completed: March 10, 2026

FOLLOW UP

Early Follow up (within 1-3 days) after disclosure

Patient Name : Zavia Beds Recipient Name: Zhara Beds Relation Sister Contact # 000191111

1	Did Zhara Beds (recipient name) express any negative reactions to the disclosure?	Yes (No)
2	Were there any unanswered questions from the disclosure?	Yes (No)

If Yes, what were those questions? State and list recommended action.

Question 1:

Action:

Question 2:

Action:

3	Does the PLHIV / Recipient require additional support?	Yes (No)
3a	If Yes, what additional support is required?	
	iv. HIV/ SRH Education	Yes (No)
	v. Counselling sessions	Yes (No)
	vi. **Other** (state name of the service):: _____	Yes (No)

Comments: Zhara expressed positive attitude towards Zavia. She says he should have come to her about it from before and that is willing to support him. She has also promised not to share the information with their parents.

HCP signature _____SW Davis_____ Date completed: March 18, 2026_____

Later/Routine Follow up (1-6 months) after disclosure.

Patient Name : Zavia Beds Recipient Name: __Zhara Beds _Relation_Sister_Contact #: 000191111_

1	Has _____Zhara_Beds_____ (recipient name) expressed any negative reactions to the disclosure?	Yes (No)

a. If Yes, briefly describe the effects of the negative reaction(s):

b. If Yes, list recommended actions: N/A

Action 1:

Action 2:

2	Does the PLHIV / Recipient require additional support?	Yes (No)

c. **If Yes, what additional support is required?** N/A

i. **HIV/ SRH Education** Yes (No)

ii. **Counselling sessions** Yes (No)

d. **Other** (state name of the service)**:** Yes (No)

_____

Comments: Zhara has continued to show positive attitudes towards Zavia and she has kept confidential the information.

HCP signature _____SW Davis_____ Date completed: April 18, 2026_____

THE USE OF ROLE PLAYS

A role play is a simulation in which each participant is given a character or position to play. Participants are given some information related to description of the role, concerns, objectives, responsibilities, emotions, etc. Then, a general description of the situation, and the problem that each one of them faces, is given (12).

Role playing can be an effective technique in preparing clients for disclosure. It allows the clients to assume a particular role and dramatize possible real-life situations in a safe space. Role plays are particularly useful for discussing sensitive and controversial issues by enabling clients to assess how they feel about these issues.

Role plays provide several benefits for the disclosure process as they:

1. Can motivate clients towards behavior change
2. Facilitate the analysis of real-world experiences
3. Enhance communication, problem-solving and negotiation skills (13)

Based on whether the topic of disclosure is being discussed during a routine clinic visit with a HCP or in a larger, more educational group setting, one of the following types of role play may be used:

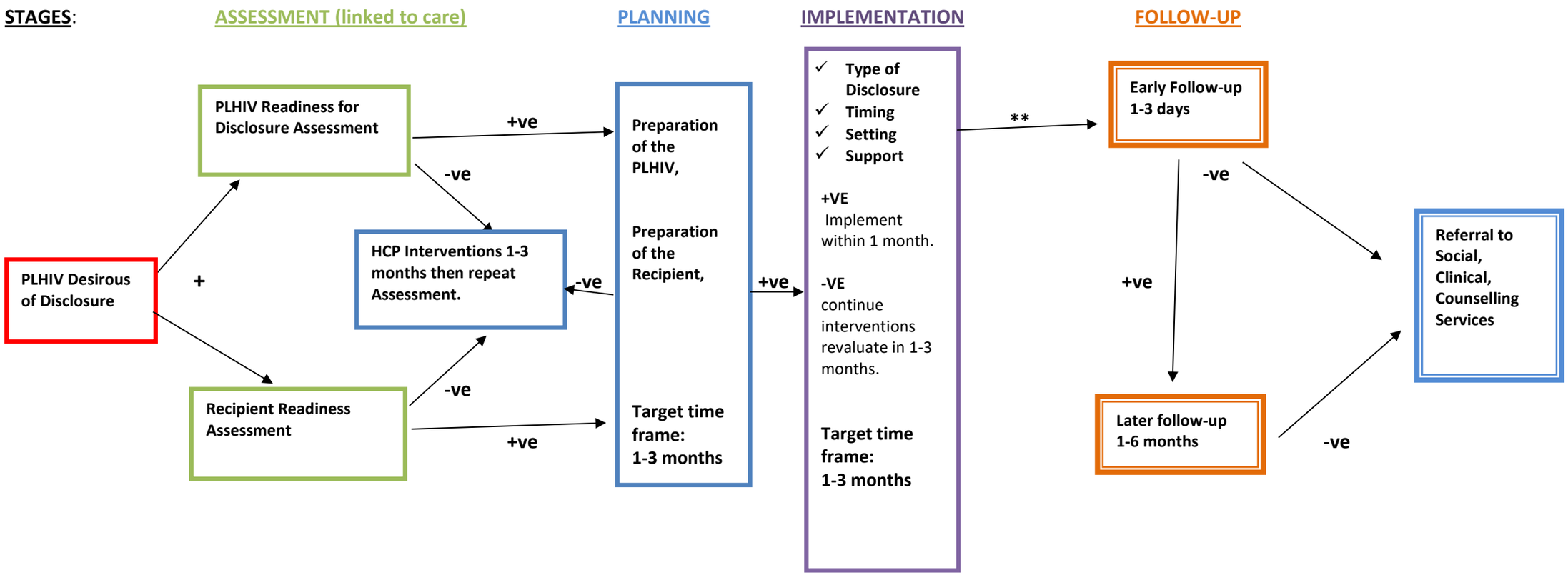
1. Spontaneous role play – allows the PLHIV to play their 'real self' in the scenario and the HCP can play the role of the potential recipient.
   This would be most useful during routine clinic visits as the HCP can play the role of a significant person in the client's life and then switch roles as necessary.

2. Single role play – entails a small subgroup enacting a given scenario for the larger group. Members of the audience in the larger group, observe and comment on the dramatization of the small group.
   The HCP may find this useful when educating a group of PLHIV about different approaches that may be taken in disclosing one's status to varied recipients.

3. Role rotation – is similar to the single role play but at some point during the activity, the HCP asks the participants to stop and discuss the role play. Afterwards, the participants are allowed to switch roles.
   The HCP may find this useful when educating a group of PLHIV on the varied perspectives/viewpoints that the potential recipients may have during and after disclosure(12).

Regardless of the type of role play used, the scenario which is enacted should be relatable and further explored by client and HCP at end through engaging discussions.

Algorithm for the Disclosure Process in Adult PLHIV



Abbreviations: -ve – negative (no), +ve – positive (yes), HCP- healthcare providers, **- If disclosure occurs. Negative Assessment is answer NO to 1 or > Assessment questions, Positive Assessment is answer YES to all Assessment questions. Negative Planning is answer NO to 1 or > Planning questions for Recipients or PLHIV, Positive Planning is answer YES to all Planning questions. Negative Implementation is answer NO to 1 or > implementation questions, Positive Implementation is answer YES to all Implementation questions. Negative Follow-Up is answer YES in Follow Up question 1, Positive Follow-Up is answer NO in Follow-up question 1.

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